DRUG AND TOXICOLOGY INFORMATION SERVICES

# DATIS

Bulletin
MAR 2025

## A NEW HOPE IN THE BATTLE AGAINST ANTIBIOTIC RESISTANCE

## PHARMACISTS DESIGNING FUTURE

## Is Pharmaceutical Compounding a Dying Practice?

Jocelyn Chaibva
President-African Pharmaceutical Forum

## Honoring Female Pharmacists in Zimbabwe

# Mr L Gadaga: Director | DATIS

Welcome to Datis Bulletin, the official publication of Drug and Toxicology Information Services (DATIS). We are dedicated to providing accurate, reliable, and up-to-date information on pharmaceuticals, toxicology, and healthcare advancements. Our mission is to empower healthcare professionals, researchers, and the public with the knowledge they need to make informed decisions and improve health outcomes.

At DATIS, we believe that sharing information is key to advancing healthcare and promoting safe and effective medication use. Through this bulletin, we aim to deliver practical insights, expert opinions, and the latest research developments. Whether it's guidance on drug safety, clinical best practices, or emerging trends in toxicology, our goal is to be your trusted source of information.

We also encourage collaboration and knowledge-sharing within our community. If you have valuable insights, articles, or experiences to share, we invite you to contribute to our bulletin. Together, we can foster a culture of continuous learning and innovation in healthcare.

Thank you for being part of our journey. We look forward to growing and learning together as we strive to make a positive impact in the world of healthcare.

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# Editor's Note

## Dear Readers,

Welcome to the March edition of Datis Bulletin! This month, we take a deep dive into a critical discussion: "Is Pharmaceutical Compounding a Dying Practice?" As the pharmaceutical industry evolves with mass production and advanced drug formulations, we explore the relevance of compounding in modern practice, the challenges it faces, and its future in patient care.

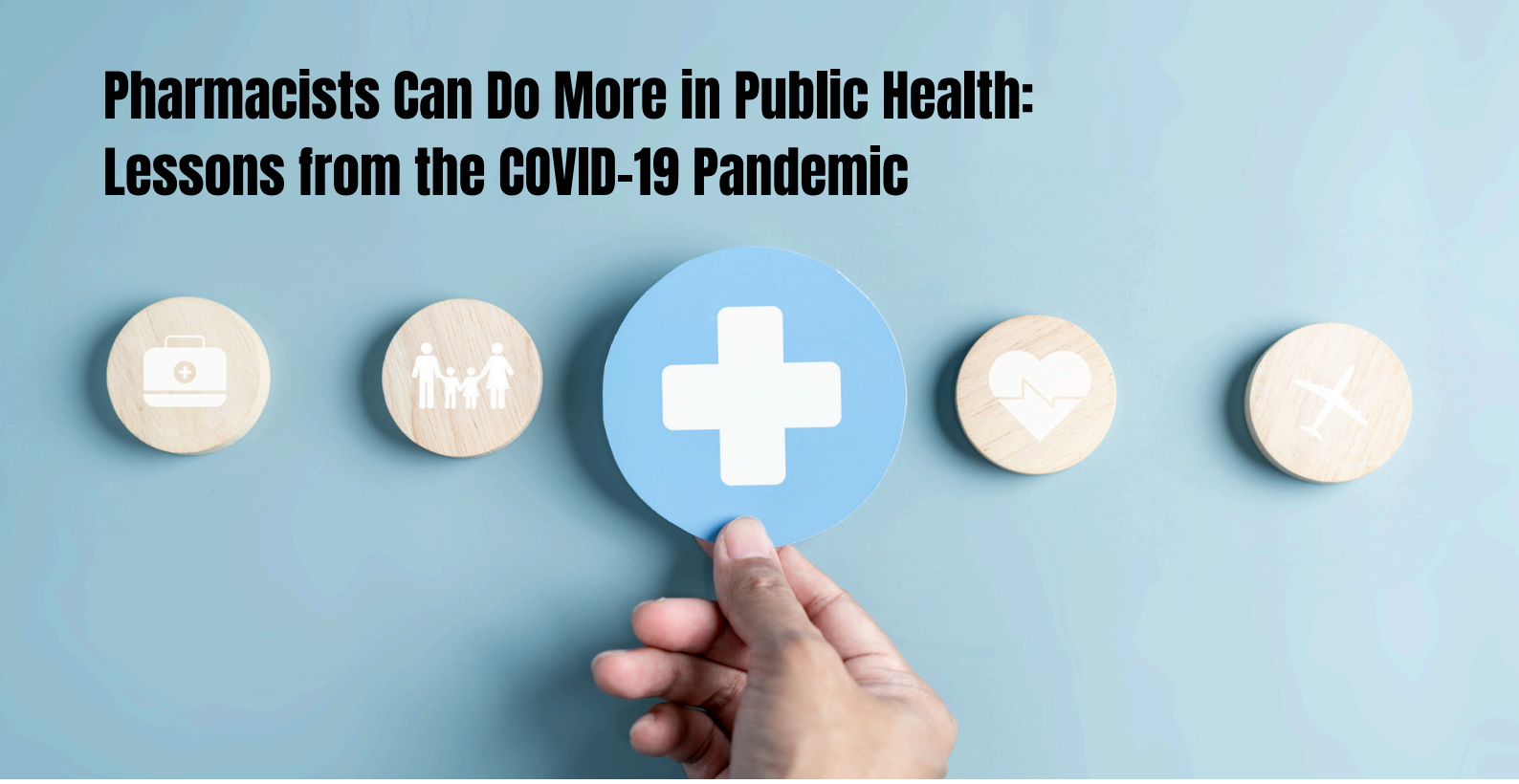
As always, we invite you to be part of the conversation! You can advertise with us to connect with a dedicated audience in the pharmaceutical and healthcare space. If you have insights, research, or experiences to share, we welcome your contributions to keep the discussion rich and impactful. Send us an email: shepardmhlaba@gmail.com

Stay engaged, stay inspired, and enjoy this edition of Datis Bulletin!

Shepard Mhlaba

Editor-in-Chief

# Pharmacists Can Do More in Public Health: Lessons from the COVID-19 Pandemic

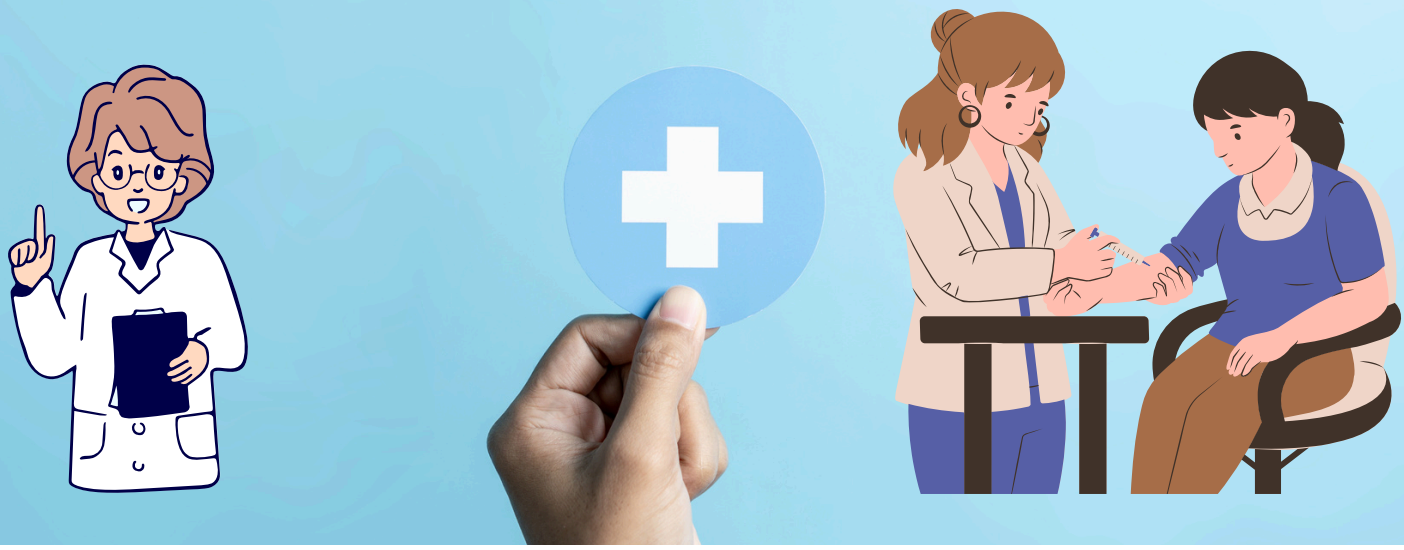


The COVID-19 pandemic highlighted the critical role of pharmacists in public health, demonstrating their ability to enhance healthcare delivery beyond traditional dispensing roles. In Zimbabwe, where public health challenges such as infectious diseases, non-communicable diseases (NCDs), and limited healthcare accessibility persist, pharmacists should be granted expanded roles in public health initiatives. This article explores why pharmacists need to be more involved in public health and outlines key areas where their expertise can significantly improve health outcomes.

## The Role of Pharmacists During the COVID-19 Pandemic

During the COVID-19 pandemic, pharmacists played a crucial role in mitigating the crisis through various activities, including:

1. **Vaccine Administration**: Many countries authorized pharmacists to administer COVID-19 vaccines, increasing vaccine coverage and accessibility (FIP, 2021).
2. **Public Health Education**: Pharmacists provided evidence-based information on preventive measures, combating misinformation and vaccine hesitancy (WHO, 2020).
3. **Screening and Testing**: Community pharmacies served as accessible points for COVID-19 testing and symptom screening (CDC, 2021).
4. **Ensuring Medicine Supply**: Pharmacists managed medicine stocks, preventing shortages and ensuring continued access to essential medicines (World Bank, 2022).
5. **Telehealth and Remote Consultations**: With movement restrictions in place, pharmacists offered virtual consultations to manage chronic diseases and medication adherence.



These contributions underscore the need for permanent integration of pharmacists into public health programs beyond emergency response situations.

## Key Public Health Roles for Pharmacists in Zimbabwe

Pharmacists in Zimbabwe can enhance the public health system by contributing in the following areas:

**1. Disease Prevention and Health Promotion**

Pharmacists are well-positioned to lead health promotion campaigns on issues such as:

- Vaccination programs for diseases like HPV, influenza, and COVID-19 (WHO, 2020).
- Smoking cessation and substance abuse counseling.
- Nutritional advice and weight management programs (CDC, 2021).
- Sexual and reproductive health education

**2. Management of Non-Communicable Diseases (NCDs)**

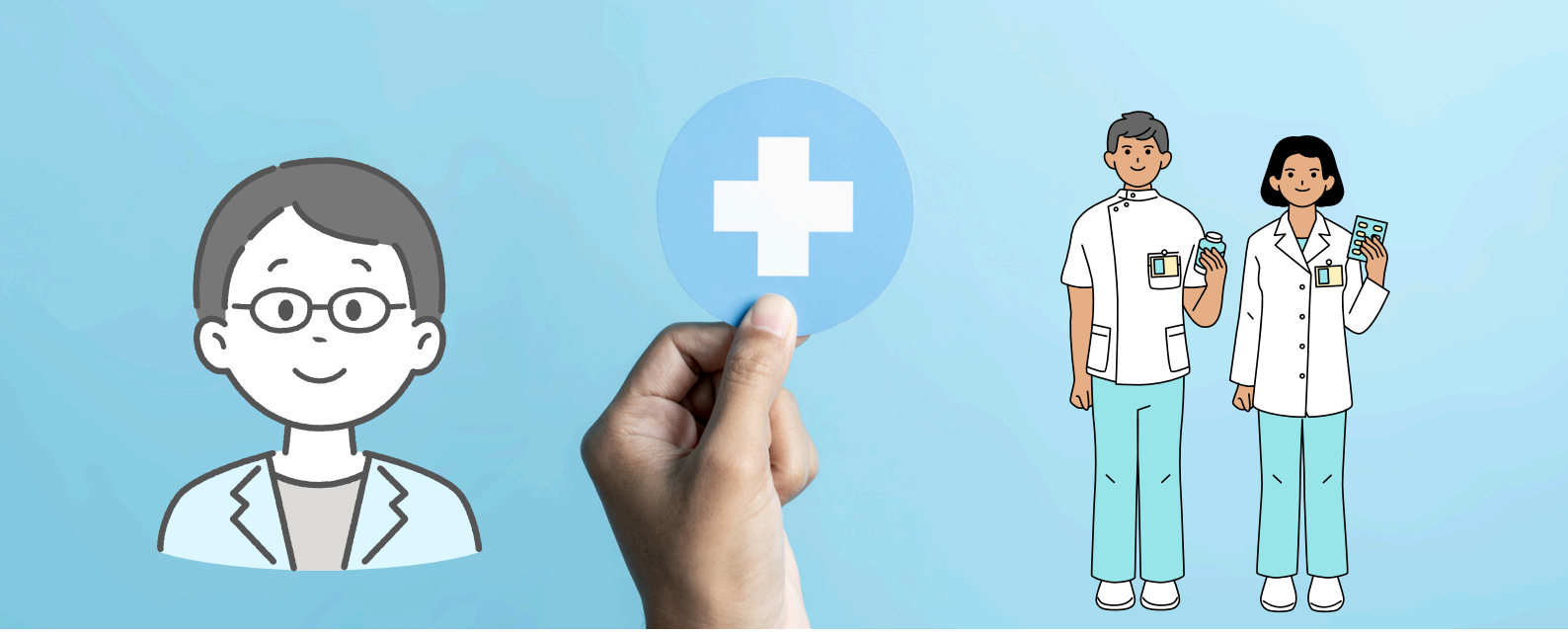
Zimbabwe is experiencing a rise in NCDs such as hypertension, diabetes, and cardiovascular diseases. Pharmacists can contribute by:

- Conducting routine screenings and early detection of NCDs (World Bank, 2022).
- Offering medication therapy management (MTM) services to optimize treatment outcomes (FIP, 2021).
- Educating patients on lifestyle modifications to prevent disease progression (MoHCC, 2021).

**3. Antimicrobial Stewardship (AMS)**

With rising antimicrobial resistance (AMR) in Zimbabwe, pharmacists should be empowered to:

- Educate the public on the dangers of antibiotic misuse.
- Guide healthcare providers on appropriate antibiotic prescribing (WHO, 2020).
- Monitor antibiotic consumption trends and contribute to national AMS policies (FIP, 2021).



**4. Maternal and Child Health Support**

Pharmacists can improve maternal and child health through:

- Counseling on prenatal vitamins and medication safety during pregnancy (MoHCC, 2021).
- Providing pediatric medication dosing guidance (CDC, 2021).
- Participating in immunization programs for children (WHO, 2020).

**5. Pharmacovigilance and Drug Safety Monitoring**

To enhance medication safety, pharmacists can:

- Report and track adverse drug reactions (ADRs) to improve pharmacovigilance systems (FIP, 2021).
- Educate patients on potential side effects and drug interactions (World Bank, 2022).
- Assist in post-marketing surveillance of medicines

**6. Emergency Preparedness and Disaster Response**

Beyond pandemics, pharmacists can assist in:

- Stockpiling and distributing essential medicines during natural disasters
- Offering first aid and trauma care training to communities (WHO, 2020).
- Supporting mental health interventions during crises (FIP, 2021).

Pharmacists have proven their ability to significantly contribute to public health, as evidenced by their role during the COVID-19 pandemic. By allowing pharmacists to take on expanded public health responsibilities, Zimbabwe can enhance healthcare accessibility, improve disease prevention efforts, and strengthen the overall health system. Policymakers must take decisive action to integrate pharmacists into national public health initiatives, leveraging their expertise to build a healthier and more resilient nation.

# GEPOTIDACIN: A NEW HOPE IN THE BATTLE AGAINST ANTIBIOTIC RESISTANCE

BY FACKWELL MATEO, BPHARM(HONS)

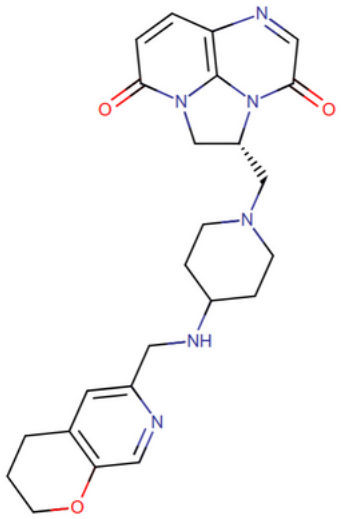
**Introduction**

Gepotidacin is a novel antibiotic and the first in its class of triazaacenaphthylene antibiotics, currently being developed by GlaxoSmithKline (GSK). With the alarming rise of antimicrobial resistance, particularly among Gram-negative bacteria, traditional antibiotics are becoming less effective in treating infections. Gepotidacin presents a promising solution in the fight against multidrug-resistant strains.

**The Need for New Antibiotics**

The global surge in antimicrobial resistance poses a significant public health threat. A major concern is resistance in Gram-negative bacteria such as Neisseria meningitidis, Klebsiella pneumoniae, Pseudomonas aeruginosa, Enterobacterspp., and Escherichia coli (E. coli), with E. coli ranked second in bacterial infection-related mortality. Among the most common infections caused by Gram-negative bacteria are urinary tract infections (UTIs) and urogenital gonorrhea. Treatment options for these infections are increasingly limited due to rising resistance.

Current treatment regimens for resistant infections often involve parenteral antibiotics, which are less convenient, more costly, and may require hospitalization. This highlights the urgent need for safe and effective oral antibiotic alternatives like gepotidacin.




**What is Gepotidacin?**

Gepotidacin is an orally administered antibiotic with a unique chemical structure belonging to the triazaacenaphthylene class. It offers a novel mechanism of action, distinguishing it from existing antibiotics.

**Mechanism of Action**

Gepotidacin inhibits bacterial DNA replication by targeting DNA gyrase and topoisomerase IV—essential enzymes for bacterial DNA replication and repair. Specifically, it selectively inhibits the B subunit of DNA gyrase and topoisomerase IV, exerting a bactericidal effect.

Although fluoroquinolones, such as ciprofloxacin, target similar enzymes, in vitro studies have demonstrated that gepotidacin remains active against fluoroquinolone-resistant bacterial strains.

## Clinical Trials and Results

### UTI Trials

Two Phase 3 clinical trials, EAGLE-2 and EAGLE-3, compared the efficacy and safety of gepotidacin with nitrofurantoin in treating uncomplicated urinary tract infections (uUTIs). Patients received either:

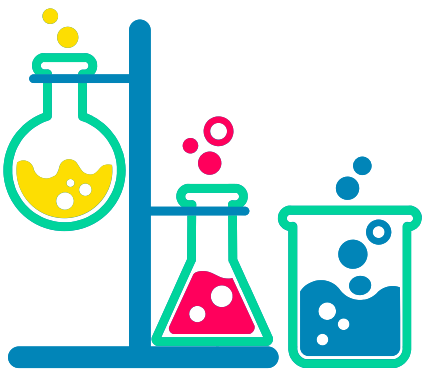
- Gepotidacin 1500 mg twice daily for five days, or
- Nitrofurantoin 100 mg twice daily for five days.

Results demonstrated that gepotidacin was non-inferior in one trial and superior in the other, confirming its effectiveness in treating uUTIs with an acceptable safety profile.

### Gonorrhea Trials

A Phase 2 clinical trial enrolled 69 adult participants who were randomized to receive either 1500 mg or 3000 mg of a single oral dose of gepotidacin. Treatment success rates were 97% and 95% for the 1500 mg and 3000 mg groups, respectively. The most common adverse effects reported included diarrhea, flatulence, nausea, and abdominal pain, occurring in 52% of patients in the 1500 mg group and 64% in the 3000 mg group.

A Phase 3 clinical trial, EAGLE-1, compared gepotidacin to the standard combination therapy of ceftriaxone (500 mg intramuscularly) plus azithromycin (1 g orally) for gonorrhea treatment. Patients in the gepotidacin group received an initial oral dose of 3000 mg, followed by a second dose of 3000 mg 10–12 hours later. Follow-up visits were conducted on days 4–8 and 14–21. Results showed that gepotidacin was non-inferior to ceftriaxone plus azithromycin, confirming its potential as an alternative treatment for gonorrhea, especially in cases of antibiotic resistance.



## Safety Profile

From clinical trials, the most commonly reported adverse effects were gastrointestinal-related, including diarrhea, flatulence, nausea, and abdominal pain. No significant safety concerns have been identified to date, but long-term surveillance will be required to fully establish its safety profile.

## Potential Impact

The discovery of gepotidacin in a new antibiotic class could pave the way for further research and development of triazaacenaphthylene antibiotics.

**Key advantages include:**

- Oral administration: Offers a convenient alternative to injectable antibiotics, particularly for patients with needle phobia or allergies to cephalosporins.
- Reduced patient waiting time: Eliminates the need for reconstitution and administration by healthcare providers, as patients can take the medication at home.
- Potential for combination therapy: Gepotidacin may be used alongside other antibiotics to combat multidrug-resistant pathogens.

## Challenges and Future Directions

Despite its promise, several challenges remain:

- Regulatory approval: Gepotidacin is still undergoing review by regulatory authorities, and approval is not guaranteed.
- Cost and accessibility: Once approved, initial pricing and patent protection may limit accessibility, particularly in low- and middle-income countries.
- Antibiotic stewardship: To prevent resistance, appropriate use of gepotidacin is crucial. Continued antimicrobial stewardship efforts will be necessary to ensure its long-term efficacy.

Additionally, given its strong activity against Gram-negative bacteria, gepotidacin holds potential for treating other infections such as respiratory tract infections, intra-abdominal infections, biliary tract infections, and brain abscesses.

**Conclusion**

The discovery of gepotidacin underscores the importance of ongoing research in antibiotic development to combat antimicrobial resistance. Bringing new antibiotics to market requires collaboration between governments, regulatory bodies, and pharmaceutical companies, all while ensuring patient safety.

As healthcare professionals, our role is to prescribe and dispense antibiotics responsibly. Educating patients on antimicrobial resistance is equally critical in safeguarding these vital drugs. By practicing prudent antibiotic use, we can help preserve the efficacy of antibiotics—including the promising future of gepotidacin.

**References**

1. Wagenlehner, F., Perry, C. R., Hooton, T. M., et al. (2023). "Efficacy and safety of oral gepotidacin in the treatment of uncomplicated urinary tract infection: results of two randomized, multicentre Phase 3 trials (EAGLE-2 and EAGLE-3)." JAC-Antimicrobial Resistance, 5(Supplement_3), dlad077.035. https://doi.org/10.1093/jacamr/dlad077.035
2. Perry, C. R., Scangarella-Oman, N. E., Millns, H., et al. (2023). "Efficacy and Safety of Gepotidacin as Treatment of Uncomplicated Urogenital Gonorrhea (EAGLE-1): Design of a Randomized, Comparator-Controlled, Phase 3 Study." Infect Dis Ther, 12, 2307–2320. https://doi.org/10.1007/s40121-023-00862-6

# Is Pharmaceutical Compounding a Dying Practice in Zimbabwe?

**Introduction**

Pharmaceutical compounding—the art and science of preparing personalized medications—has played a vital role in healthcare for centuries. However, in recent years, its prevalence has significantly declined worldwide. This trend is particularly noticeable in Zimbabwe and the broader Southern African region. While compounding remains essential in certain medical scenarios, several economic, regulatory, and practical challenges have led many pharmacists to step away from this practice. This article explores the current state of pharmaceutical compounding, factors contributing to its decline, remuneration models, and strategies to revitalize this crucial aspect of pharmacy practice.

**The Decline of Pharmaceutical Compounding**

Historically, nearly half of all medical prescriptions were compounded by pharmacists, addressing specific patient needs that mass-produced medications could not meet. Today, this figure has decreased dramatically due to the rise of commercial pharmaceutical products, which offer convenience and standardization. In Zimbabwe, the economic instability and limited local pharmaceutical manufacturing capabilities further exacerbate this decline. Additionally, the focus on generic drug production has shifted attention away from personalized medications, reducing the demand for compounding services.

**Challenges Faced by Pharmacists**

Several key challenges have contributed to the waning interest in pharmaceutical compounding:

**1. Resource Limitations:**

Compounding requires specialized equipment, high-quality raw materials, and rigorous quality control processes. In resource-constrained environments like Zimbabwe, obtaining and maintaining these essentials is often difficult.

**2. Regulatory Hurdles:**
The absence of comprehensive regulatory frameworks makes it challenging to ensure the standardization and quality of compounded medications. This lack of oversight can lead to legal risks and concerns about product safety.

**3. Economic Constraints:**
Given the modest remuneration for pharmacists in Zimbabwe—averaging approximately \$6.62 per hour or between \$12,000 to \$23,000 annually—the time and effort required for compounding may not be financially viable. Many pharmacists may prefer to focus on dispensing commercially available medications that are more profitable and less time-consuming.

**4. Professional Training and Confidence:**
Limited opportunities for specialized training in compounding can reduce pharmacists' confidence and competence in this area, further discouraging its practice.

**Remuneration Models for Compounding Pharmacists**
The financial viability of pharmaceutical compounding depends largely on remuneration models that fairly compensate pharmacists for their time, expertise, and resources. In regions where compounding is still prevalent, pharmacists typically charge a fee that reflects the complexity of the preparation, the cost of raw materials, and the time involved. However, in Zimbabwe, the current compensation structure may not adequately reward the additional effort required for compounding, making it less attractive as a professional pursuit.

**Strategies to Revitalize Pharmaceutical Compounding**
To reverse the decline of pharmaceutical compounding and harness its benefits for patient care, several measures can be implemented:

**1. Develop Clear Regulatory Guidelines:**
Establish comprehensive regulations to standardize compounding practices, ensuring product quality and patient safety while reducing legal risks for pharmacists.

**2. Enhance Training and Education:**
Integrate compounding modules into pharmacy curricula and provide continuing professional development opportunities. Specialized training programs can boost pharmacists' confidence and competence, encouraging more professionals to engage in compounding.

**3. Introduce Economic Incentives:**
Offer financial incentives such as subsidies for raw materials, tax breaks, or reimbursement schemes for compounded medications. These measures can make compounding more financially viable, attracting more pharmacists to the practice.

**4. Invest in Infrastructure:**
Allocate resources to improve access to high-quality raw materials and modern equipment, enabling pharmacists to produce safe and effective compounded medications.

**5. Foster Professional Collaboration**: Establish or strengthen professional associations dedicated to compounding, such as the

# Is Pharmaceutical Compounding a Dying Practice in Zimbabwe?

Association of Compounding Pharmacists of South Africa (ACPSA). These organizations can provide support, advocacy, and networking opportunities for pharmacists interested in compounding.

While pharmaceutical compounding has experienced a decline in Zimbabwe and Southern Africa, it remains a vital component of personalized healthcare. Addressing the challenges faced by pharmacists through regulatory support, enhanced training, economic incentives, and improved infrastructure can help revitalize this essential practice. By embracing these measures, the region can ensure that patients continue to benefit from tailored medications that meet their unique needs, ultimately enhancing the quality of healthcare available to all.

**“Recognition of compounding services as an incentive which is billable and reimbursable to pharmacists is key”**

# PHARMACISTS DESIGNING FUTURE

**Designing Your Future: How Pharmacists Can Apply Life Design Principles to Their Careers**

In the fast-evolving world of healthcare, pharmacists face numerous challenges—from adapting to digital transformations to navigating regulatory changes. Bill Burnett and Dave Evans' book, Designing Your Life, offers a refreshing perspective on how pharmacists can apply design thinking to both their personal and professional journeys. By integrating these principles, pharmacists can build fulfilling careers while maintaining work-life balance and professional satisfaction.

## 1. Reframing Pharmacy Careers as a Design Challenge

Pharmacists often feel constrained by traditional career paths, whether in retail, hospital, or industry settings. Burnett and Evans encourage individuals to treat their careers as design challenges rather than linear paths. By embracing this mindset, pharmacists can explore alternative roles such as digital health, consultancy, or entrepreneurship.

## 2. Prototyping Career Options

A core principle of Designing Your Life is prototyping—testing career choices before making long-term commitments. Pharmacists can apply this by:

- Shadowing professionals in non-traditional roles like pharmaceutical logistics or regulatory affairs.
- Taking short-term courses in emerging fields such as telepharmacy or artificial intelligence in healthcare.
- Volunteering in public health initiatives to explore new interests.

## 3. The Power of "Wayfinding"

Wayfinding is about making career decisions without a rigid long-term plan but rather through continuous small steps. Pharmacists can use this approach by:

- Attending industry conferences to identify future trends.
- Engaging in networking and mentorship opportunities.
- Continuously upskilling through certifications and workshops.

## 4. Work-Life Balance Through Energy Mapping

Pharmacists often face burnout due to long hours and demanding environments. Burnett and Evans suggest using "energy mapping" to identify tasks that energize or drain an individual. Pharmacists can apply this by:

- Delegating administrative tasks where possible.
- Seeking roles that align with their interests, such as patient counseling for those who enjoy direct interaction.
- Incorporating mindfulness and wellness practices into daily routines.

## 5. Failing Forward: Embracing Setbacks

Failure is often seen negatively, but Designing Your Life teaches that setbacks are opportunities for growth. Pharmacists who have faced challenges—whether in business ventures, failed job applications, or workplace issues—can use these experiences to refine their career paths.

## 6. Building a Supportive Community

The authors emphasize the importance of "co-designing" life with others. Pharmacists can benefit from:

- Joining professional associations and networking groups.
- Seeking mentorship from experienced colleagues.
- Collaborating with interdisciplinary teams to enhance patient care.

# The Impact of Long Working Hours on Pharmacists: Balancing Professional Duties and Personal Well-being

Pharmacists play a pivotal role in healthcare, ensuring the safe and effective use of medications. However, the demanding nature of their profession—characterized by long hours, shift work, and high-pressure environments—can significantly impact both their professional performance and personal well-being.

**Professional Implications of Extended Working Hours**

Extended working hours have been linked to increased stress levels among pharmacists. A study highlighted that 71.7% of community pharmacists reported heightened stress over the past year, attributing this to increased work volume (75.2%) and heightened management pressures (48.4%) . Such stress can lead to burnout, characterized by emotional exhaustion, depersonalization, and reduced personal accomplishment, ultimately diminishing job satisfaction and productivity.

Moreover, prolonged working hours can compromise patient safety. Fatigue resulting from extended shifts impairs cognitive functions, increasing the likelihood of dispensing errors and negatively affecting patient care quality. Research indicates that pharmacists working over 40 hours per week report lower job satisfaction and higher work-life conflict .

**Personal Consequences of Long Working Hours**

The personal repercussions of extended working hours are profound. Pharmacists often experience reduced family time, leading to strained personal relationships and social isolation. The imbalance between professional responsibilities and personal life can also result in increased stress levels and decreased job satisfaction . Over time, this imbalance may contribute to mental health issues such as anxiety and depression.

Physical health is also at risk. Chronic occupational stress has been associated with various health problems, including cardiovascular diseases and musculoskeletal disorders . The demanding nature of pharmacy work, combined with insufficient rest, can exacerbate these health risks, leading to absenteeism and decreased overall well-being.

**International Best Practices for Enhancing Work-Life Balance**

Recognizing these challenges, several international best practices have been proposed to improve pharmacists' work-life balance:

1. **Flexible Scheduling**: Implementing flexible work schedules allows pharmacists to manage their professional and personal commitments more effectively, reducing stress and enhancing job satisfaction .
2. **Supportive Work Environment**: Fostering a workplace culture that promotes open communication and mutual support can alleviate stress. Employers are encouraged to work closely with regulatory bodies and other stakeholders to ensure that employment contracts do not negatively impact employees' work-life balance .
3. **Wellness Programs**: Introducing wellness initiatives, such as stress management workshops, physical activity programs, and mental health resources, can help pharmacists cope with occupational stress and maintain a healthier lifestyle .
4. **Adequate Staffing**: Ensuring sufficient staffing levels can distribute workloads more evenly, preventing burnout and maintaining high-quality patient care. Addressing workforce shortages is crucial to avoid scheduling conflicts and difficult working conditions .

By adopting these best practices, the pharmacy profession can move towards a more sustainable work environment, benefiting both pharmacists and the communities they serve.

# Community Pharmacy Section

Thinking 🤔 about how to be heard or who to approach regarding your community pharmacy practice concerns? Another reason you should join the Community Pharmacy Association (CPA).

# HONORING FEMALE PHARMACISTS IN ZIMBABWE

*Pioneers of Healthcare on International Women's Day*

International Women's Day is a time to celebrate the achievements of women across the globe and to acknowledge their contributions in various fields. In Zimbabwe, female pharmacists play an indispensable role in shaping the healthcare landscape, ensuring access to quality medicines, advocating for public health, and driving innovation in pharmaceutical practice. Today, we recognize their resilience, expertise, and dedication to advancing the pharmacy profession despite numerous challenges.

## Breaking Barriers in Pharmacy

Historically, pharmacy was a male-dominated profession, but over the years, women have broken barriers and assumed leadership roles in academia, industry, regulatory affairs, and community practice. Female pharmacists have not only excelled in their professional duties but have also contributed significantly to policy development, research, and mentorship, paving the way for future generations of women in pharmacy.

## Champions of Public Health and Patient Care

Women in pharmacy have been instrumental in spearheading initiatives that enhance patient care, medication safety, and accessibility to essential medicines. From running community pharmacies to working in hospital settings, female pharmacists ensure that patients receive appropriate pharmaceutical care. Their dedication is evident in rural and urban areas alike, where they educate communities on medication use, disease prevention, and health promotion.

## Leadership and Innovation

Despite the challenges of gender bias and limited leadership opportunities, many female pharmacists in Zimbabwe have risen to top positions in government, regulatory bodies, and pharmaceutical organizations Their leadership in institutions such as the

**“To all the incredible women in pharmacy, we honor your dedication, passion, and commitment to improving healthcare in Zimbabwe. Your contributions are invaluable, and your impact will continue to shape the future of the pharmaceutical profession for years to come. Happy International Women's Day!”**

Medicines Control Authority of Zimbabwe (MCAZ), the Pharmacists Council of Zimbabwe (PCZ), and Pharmaceutical Society of Zimbabwe (PSZ) has influenced policies that strengthen the pharmacy profession.

Moreover, Zimbabwean women in pharmacy have embraced digitalization and technological advancements to streamline supply chains, enhance pharmaceutical logistics, and improve patient outcomes. They have also played a pivotal role in research and development, contributing to innovations in pharmaceutical compounding and drug manufacturing.

## Overcoming Challenges

Female pharmacists face several challenges, including workplace discrimination, limited access to funding for entrepreneurial ventures, and the struggle to balance professional and personal responsibilities. Despite these obstacles, they continue to excel, advocating for better working conditions, mentorship programs, and policies that support gender equity in the pharmaceutical sector.

## Inspiring the Next Generation

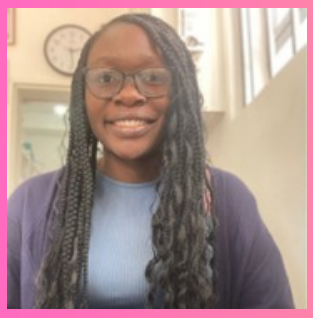
The impact of female pharmacists extends beyond their professional duties. Through mentorship programs, career guidance, and capacity-building initiatives, they inspire young women to pursue pharmacy as a career. Their involvement in training programs, such as the Zimbabwe Pharmaceutical Students Association (ZPSA) and early career Pharmacists training, ensures that the next generation of pharmacists is well-equipped to meet the demands of the profession.

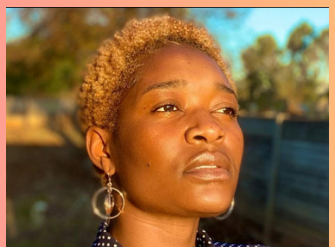
## A Call for Continued Support and Recognition

As we celebrate International Women's Day, it is essential to recognize and support the contributions of female pharmacists in Zimbabwe. The government, private sector, and professional associations should continue to promote gender equality, offer leadership training, and provide opportunities for women to thrive in pharmacy.

# International Women's Day

*Celebrating our Pharmacists*





# Mutoko High School Drug Abuse Awareness

On Tuesday 11 March 2025, representatives from the Pharmaceutical Society of Zimbabwe (PSZ) and representatives from the Zimbabwe Pharmaceutical Students Association (ZPSA) visited Mutoko High Secondary School, to raise awareness on the dangers of drug and substance abuse amongst the students.

They addressed to more than 1 052 students and, 10 school staff members present.

PADA
Pharmacists Against Drug Abuse

**The team comprised of:**

Samantha Mapani 3.2 (HIT)
Tadiwa Maposa 1.2 (UZ)
Tanyaradzwa Tabengwa 1.2 (HIT)
Rutendo Chingamuka 1.2 (UZ)
Nicholas Muparadzi (Pharmacist)

**Let's join forces in the fight against drug and substance abuse!**

FIRST MUTUAL PHARMACIES

The Pharmaceutical Society of Zimbabwe

PADA Pharmacists Against Drug Abuse

# REHABILITATION CENTERS IN ZIMBABWE

COMPILED BY THE

## PHARMACISTS AGAINST DRUG ABUSE

***VERSION 1.1 OF 2025***

| | | |
|---|---|---|
| **Care Addictions Rehab, Bulawayo** | + 263 775424284 | Located in Bulawayo. |
| **For Youths By youths , Harare & Mashwest** | +263 775462468 | Inpatient therapy, support and supervised medical detoxification. Open for collaborations. |
| **Healing Rain Rehab, Vumba** | +263 779308145 | They shut down. But now they have support groups that help parents and guardians of addicts. They sent a video we can use at our PADA website. Request Vincent if the video is needed. Open for collaborations |
| **Beatrice Rehabilitation** | +263776 991153 | Located in Harare. |
| **Tariro Rehabilitation Centre** | +263712 799605 | Located in Harare. |
| **Active Youth Zimbabwe, Mpumula** | + 263779728662 | Located in Bulawayo. |
| **Youth with a mission** | +263 774702407 | Harare & Mashonaland West |
| **Sally Mugabe Psychiatric Unit** | (024)2621100 | Located in Harare. |
| **Parirenyatwa Annex Psychiatric Unit** | +263 242 7015943/242 701623 | Located in Harare. |
| **Inguthseni Psychiatric Hospital** | (029) 2473896 | Located in Bulawayo. |
| **Ngomahuru Hospital and Halfway House, Masvingo** | +263 966463 | Located inMasvingo. |

**IMPORTANT CONTACT DETAILS:**
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**Pharmacists Against Drug Abuse (PADA): +263 772 143 333**
**Drug and Substance Abuse (DSA) National Toll Free: +263 714 647 002**
**ZRP CID Anonymous Tip Off (Whatsapp): +263 712 800 197**
**ZRP Landline: (0242) 700171-9 / (0242) 771874**

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| Centre Name | Contact information | Extra information |
|---|---|---|
| **Borrowdale halfway house** | +26378858831 (same number for WhatsApp) | Psychiatric rehabilitation, drug abuse, alcohol abuse, sexual abuse, mental problems |
| **Mandipa Home Rehabilitation** | +263 776991153/5 ( same number for WhatsApp) | Counselling , detoxing, housing. They first conduct Tests and assessments then takes the patient to Doctors. Rehabilitation is usually a minimum of 2 months depending on the condition. |
| **Gateway Mental Health Rehabilitation** | +263 788080001 | Offer counselling( HIV, marital affairs, drug and substance abuse, clinical psychiatry. Minimum rehabilitation of a month$80 consultation |
| **Tov nation centre for addiction and recovery support** | +263 783627742 | Outpatient, bootcamps6 weeks minimum rehabilitation period |
| **Ruwa Rehabilitation centre** | +263779816966 | Physical rehab, drug abuse rehab can be requested |
| **Serenity Mind Centre** | +263 716825245 | Offer alcohol and substance abuse issues, |
| **Tirivanhu Therapeutic centre** | +263 773 593014 | Mental health issues |
| **Healthy Lifestyle Institute, Kwekwe** | +263 772 772 913 | Detox therapy, occupational therapy, cognitive behavioural therapy mental health rehabilitation services.They offer none synthetic supplements to patients. Recovery is usually 28 days |
| **Mubatirapamwe Trust** | +263 772379751 | Located in Harare. |
| **Chipadze Rehabilitation Centre** | +263778518160 | Located in Bindura. |

**In Partnership With**

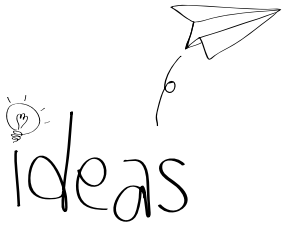
Pharmacists Against Drug Abuse

12th IPSF AFRICAN SYMPOSIUM

nm podcasts

# WHY YOU SHOULD ATTEND THE FIP CONGRESS

Attending the International Pharmaceutical Federation (FIP) World Congress offers an unparalleled opportunity for pharmacists, pharmaceutical scientists, and educators to engage with a global network of professionals dedicated to advancing healthcare. The upcoming 83rd FIP World Congress, themed "Pharmacy Forward: Performance, Collaboration, and Health Transformation," will take place in **Copenhagen, Denmark, from 31 August to 3 September 2025.**




Log on to www.fip.org to register for the congress and membership

This congress will delve into the evolving role of the pharmacy profession in addressing modern healthcare challenges, focusing on enhancing healthcare system performance, fostering cross-sector collaboration, and driving transformative health outcomes globally.

## "Expand Professional Networks"

Attending the FIP World Congress is not just about professional development; it's about being part of a global movement dedicated to **improving health outcomes worldwide**. Don't miss this chance to be part of a transformative experience that will shape the future of pharmacy and healthcare.

# VISIT COPENHAGEN DENMARK

13th IPSF AFRICAN PHARMACEUTICAL SYMPOSIUM
Harare, Zimbabwe

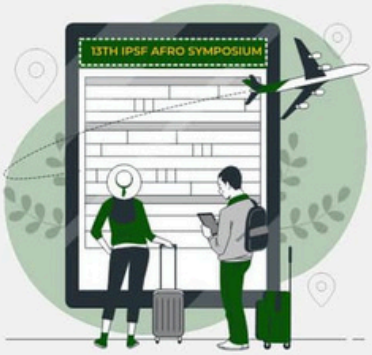
International Pharmaceutical Students' Federation
African Regional Office

# Theme

**Transforming Healthcare:** *Advancing Pharmacy Practice through Digital and Medicinal Innovations, Patient-Centered Care, and Healthcare Systems Strengthening in Africa.*

## ABOUT...

The 13th International Pharmaceutical Students Federation African Pharmaceutical Symposium (IPSFA Symposium) is a platform for pharmacy students and early career pharmacists and pharmaceutical scientists across Africa to engage with leading experts, share knowledge, and discuss innovative solutions to the challenges facing healthcare in the continent. This symposium aims to foster collaboration, inspire future leaders, and contribute to the development of a robust and resilient African healthcare system.




- *Leaders in Training: 3 DAYS*
  *HARARE*
- *Main Symposium: 5 DAYS*
  *HARARE*
- *Post Symposium Tour: 3 DAYS*
  *MUTARE, NYANGA, MASVINGO*

**ZIMBABWE 2025**

*TO PARTNER, CONTACT:*

*Tapiwa: +263 7 77225693 / tchinyepe@13thafps.co.zw*
*Nicodimus: +263 7 82932925 / chairperson@13thafps.co.zw*

13TH AFRICAN PHARMACEUTICAL SYMPOSIUM

Theme

Transforming Healthcare: Advancing pharmacy practice through digital and medicinal innovations, patient-centered care, and healthcare systems strengthening in Africa

ABOUT IPSF AFPS

The 13th International Pharmaceutical Students Federation African Pharmaceutical Symposium (IPSF AfPS) is a platform for pharmacy students, early career pharmacists and pharmaceutical scientists across Africa to engage with leading experts, share knowledge, and discuss innovative solutions to the challenges facing healthcare in the continent.

This symposium aims to foster collaboration, inspire future leaders, and contribute to the development of a robust and resilient African healthcare system.

LEADERSHIP IN TRAINING: 3 DAYS, HARARE
MAIN SYMPOSIUM: 5 DAYS, HARARE
POST SYMPOSIUM TOUR: 3 DAYS, (MUTARE & NYANGA)

13th IPSF AFRICAN PHARMACEUTICAL SYMPOSIUM
Harare, Zimbabwe

International Pharmaceutical Students' Federation
African Regional Office

REGISTRATION PHASES

| PHASE 1 | PHASE 2 | PHASE 3 |
|---|---|---|
| Food & Drinks<br>Accommodation<br>Symposium Material<br>Opens<br>27 November 2024<br>Closes<br>27 January 2025 | Food & Drinks<br>Accommodation<br>Symposium Material<br>Opens<br>14 February 2025<br>Closes<br>14 April 2025 | Food & Drinks<br>Accommodation<br>Symposium Material<br>Opens<br>27 May 2025<br>Closes<br>27 June 2025 |
| $260 | $280 | $300 |

LIT: $100 - PST $150

LIMITED SEATS AVAILABLE

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REGISTER NOW

MORE INFORMATION
www.13thafps.co.zw
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Visit Zimbabwe!

# JOINT DATIS & PSZ SYMPOSIUM

## Theme: Pharmacy Regulation

Free entry!

### Workshop highlights:

- 1. Clarify professional expectations and conduct for pharmacists in Zimbabwe.
- 2. Discuss the challenges currently affecting practicing pharmacists
- 3. Address the impact of brain drain on the pharmacy sector and the profession as a whole.

### Guest

Jocelyn Chaibva
**FIP-APF President**

### What you can get:

- CE Certificates
- Seminar Materials
- Lunch

## 22 MARCH 2025

Medical Lecture Theatre,
Department of Pharmacy,
University of Zimbabwe

From 11.00 AM - 0200 PM

Contact/Call:0772854552 or 0772143333

DPS HPA PCZ MCAZ PSZ PMA CPA PWA ZPSA

# March Zoom CPD

## Datis CPD Notice Board

MARCH ZOOM MEETING

**Panelists from:**

**HPA, PCZ, PSZ, MCAZ, PMA, PWA, CPA, ZPSA**

**Topic**:
**Regulation of Pharmacy in Zimbabwe**

https://us06web.zoom.us/j/83920842017?pwd=xGj1QJV00g6vggEf4K2HS7aPyqAn9N.1

**Meeting ID: 839 2084 2017** **Passcode: 048851**

# EDITORIAL TEAM

DIRECTOR

**Louis Gadaga** is the Chairperson of the Department of Pharmacy and Pharmaceutical Sciences at University of Zimbabwe. He is a lecturer in the Pharmaceutics and Clinical Pharmacy section. He is the Director of the Drug and Toxicology Information Service and he is a member of various societies. His research interest is in neuropharmacology, toxicity testing, natural products, nanotoxicology and nanomedicine, formulation and characterisation of novel oral delivery systems for the effective delivery of traditional medicines. He has also supervised over 40 undergraduate honours research students. He is currently embarking on his PhD. He has over 10 publications.

EDITOR & DESIGNER

**Shepard Nqobile Mhlaba** serves as the Global Lead for Early Career Training Strategy at the International Pharmaceutical Federation (FIP), where he plays a pivotal role in shaping pharmacy education and workforce development. He is also a journal reviewer for the FIP Pharmacy Education Journal (PEJ), contributing to the advancement of pharmaceutical research.

In addition, Shepard is the Assistant Director at the Drug and Toxicology Information Services (DATIS), where he supports drug information dissemination and toxicology expertise. His contributions to pharmacy research and innovation have been recognized through the FIP Health and Medicines Information Section (HaMIS) Travel Grant (2019) and the FIP Young Pharmacists Group (YPG) Grant for Professional Innovation (2019), underscoring his commitment to advancing the profession.

CO-EDITOR

**Nicholas Muparadzi** is currently working as a Data Support Coordinator at the International Pharmaceutical Federation (FIP) in the Global Pharmaceutical Observatory (GPO) team. Nicholas is also working as a pharmacist at the Drug and Toxicology Information Services (DaTIS), located at the Department of Pharmacy, University of Zimbabwe. Previously, Nicholas has served in the FIP ECPG Steering Committee as the Chairperson of Public Relations.

Nicholas also worked in the HaMIS, Member Communications Committee (MemCo), and was a recipient of the 2024 HaMIS congress travel grant. Nicholas is the Mashonaland/Manicaland Branch Chairperson at the Pharmaceutical Society of Zimbabwe (PSZ) and immediate past chairman of the Pharmacists Against Drug Abuse (PADA).

# DATIS Bulletin

03 EDITION
2025

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